SHISHU

Every family’s story, heard early

A multilingual WhatsApp & voice companion that helps mothers recognise inherited disorders in their children — early enough to matter.

Koita Health-a-thon 2026

THE PROBLEM

Inherited diseases and diagnostic odyssey

70 million Indians live with a rare disease, four in five of them genetic

Hemoglobinopathies, inborn errors of metabolism and of immunity, neuromuscular disorders and chromosomal syndromes make up most of the burden.

Endogamy and consanguinity concentrate the risk

Marriage within a community, sustained across generations, drives both the incidence and the prevalence of these disorders in India.

The warning signs sit across a family tree

Faltering growth, lost milestones, recurrent infections and repeated infant deaths accumulate over years, and across a pedigree nobody ever draws.

A doctor’s visit is a snapshot

Records do not follow the family and genetic tests are rarely ordered.

Most families are unaware of this risk, and some families get to know too late

After years of illness, irreversible damage, or another affected birth — by which time sometimes, a diagnosis can no longer change the outcome, only explain it.

WHY THE PATTERN IS MISSED

Where today's system loses the family history

The ASHA cannot be everywhere, and cannot ask everything

She is one worker covering scattered hamlets, and the remotest households are seen least often. Taking a family history properly takes time she does not have, and every question comes with a register to fill. History-taking is the first thing a tedious paper workload squeezes out.

Distance decides whether a family ever learns there is a problem

Families travel when a child is already very ill. Everything that should have been done — the question, the test, the counselling — simply does not happen.

So each illness is treated on its own, and the pattern is missed

Without a longitudinal history, a recurrent infection is typically another course of antibiotics. A failure to thrive child is another case of malnutrition.

Often, mothers are the first to be blamed in India

Families are quick to tell a woman that the fault is hers. A mother who expects to be blamed does not volunteer that two babies died before their first birthday, or that she and her husband are cousins — the two facts that would have changed the outcomes.

Who is SHISHU for?

Mother — the one person who has seen every episode

She has been at every admission. She is also the only one who knows about this child, the previous ones and even similar history in the families.

NOT

- A diagnostic tool
- A replacement for a genetic counsellor
- A second opinion
- A frontline health worker aide

WHAT WE ARE BUILDING

SHISHU — a conversation, not a questionnaire

A multilingual voice assistant that talks with a mother about her family's health history and what she is worried about now — in her own language, on WhatsApp or an ordinary phone call.

Listens

Natural conversation about family health history and present concerns. Questions a mother can answer from what she has seen, not a form to fill.

Explains

Culturally appropriate curated education about the conditions that run in families, and what their early signs look like in a child.

Reaches her & follows up

WhatsApp and voice calls, so it works without assuming literacy, and without another trip to a clinic. Periodically asks follow-up questions.

Resources

When a family needs more than information, it helps them reach genetic services and counsellors in their own area.

Why now?

Conversations are finally possible, and in their mother tongue

Language models can now hold multiple back-and-forth conversations in Indian languages — following what a mother says, asking the next question because of her last answer, and explaining a specific condition in words she uses herself. A printed checklist could never do this; until very recently, neither could software.

It listens where a form cannot

A questionnaire collects answers. A conversation picks up the thing said in passing — marriage to a relative, a child who died “of fever”, a baby who stopped sitting — and knows to follow it. That is the part of the history that never surfaces because it is rarely revealed and always judged.

The phone is already in the family

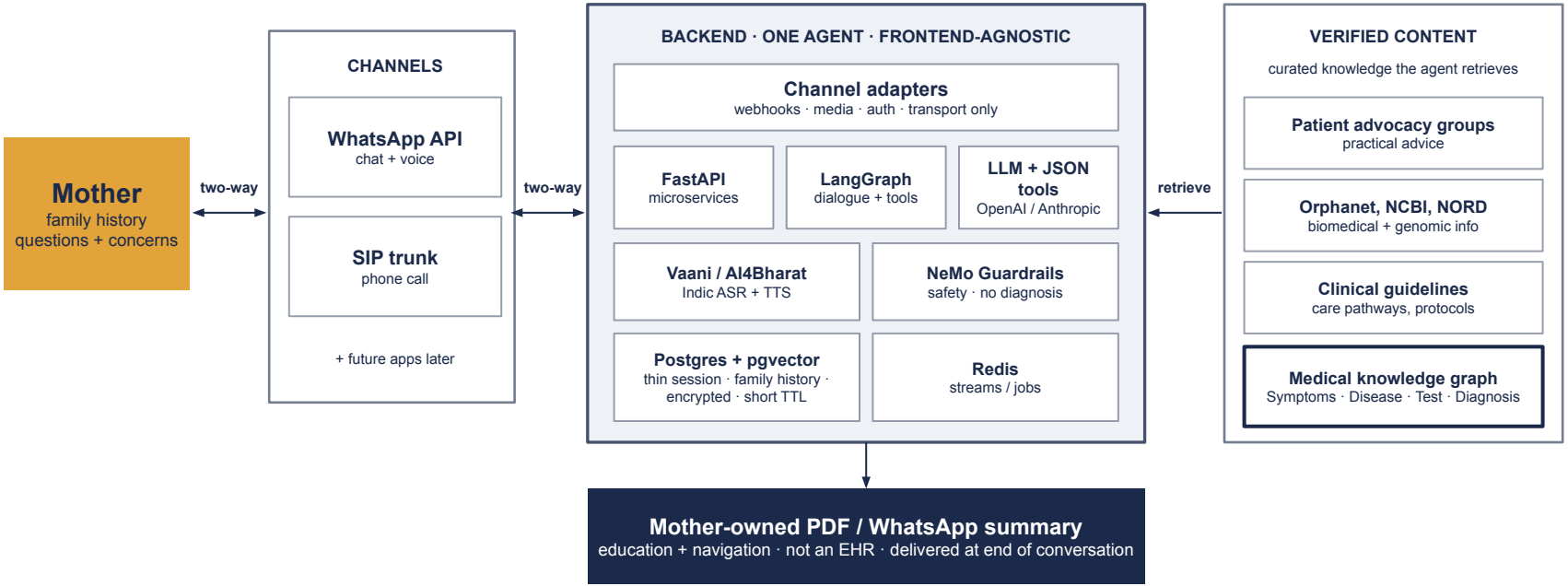
Voice calls and WhatsApp reach mothers with no literacy, no smartphone app and no second trip to a clinic. The delivery channel exists, is trusted, and costs almost nothing per family.

Digital, intelligent conversations with mothers about their children’s risks can now raise awareness and enable screening — improving access at population scale.

HOW IT WORKS

SHISHU — system architecture

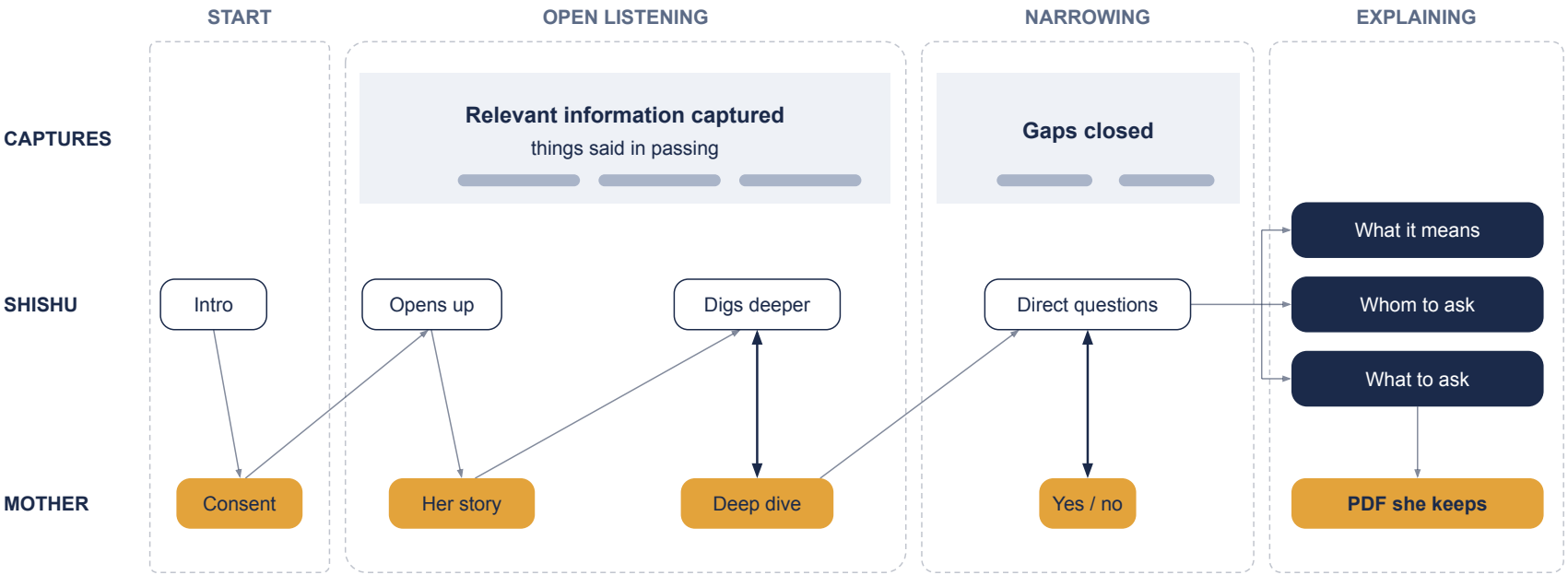
Two-way voice/chat on WhatsApp or phone → one backend agent → PDF summary to the mother



THE CONVERSATION

The conversation flow

She sets the order first. Direct questions only close what is still missing. Arrows that point both ways repeat until nothing is left hanging.



POTENTIAL IMPACT

What changes for families

1 A diagnosis that arrives early enough to matter

2 The next pregnancy becomes a choice instead of a repeat

3 Mothers stop carrying the blame